

MEGACID TABLET

Description and composition:

Megacid Tablet

Light pink coloured, circular, uncoated, beveled edged, chewable tablets with mint flavour. Diameter of the tablet 20.20mm $\pm$ 0.20mm and Thickness 4.60mm $\pm$ 0.20mm

Name(s) and strength(s) of Active Ingredient(s):

Each Chewable tablet contains:

Alginic acid	B.P	200 mg
Dried Aluminium Hydroxide	B.P	250 mg
Magnesium Hydroxide	B.P	250 mg
Magnesium Trisilicate	B.P	250 mg
Activated Dimethicone	B.P	125 mg

Pharmacology:

Alginic acid or magnesium or sodium alginates are used with an antacid or a H₂ – receptor antagonist, in the management of gastro-oesophageal reflux. The antacid or H₂ — receptor antagonist reduces the acidity of the gastric contents and the alginates reacts with gastric acid to form a viscous gel (often termed a raft) which floats on top of the gastric contents. This raft then acts as a mechanical barrier to reduce reflux.

Magnesium hydroxide is an antacid and also possesses laxative properties and is often given in conjunction with aluminium -containing antacids such as aluminium hydroxide in order to reduce their constipating effects.

Magnesium trisilicate is an antacid. The antacid action is exerted slowly, so that it does not give such rapid symptomatic relief as the alkali carbonates, bicarbonates, and oxides; however, the action is prolonged.

Aluminium hydroxide is an antacid. In order to reduce the constipating effects, it is often given in association with a magnesium -containing antacid.

Simethicone reduces the surface tension of gas bubbles, causing them to coalesce. It is used in the treatment of flatulence and meteorism, for elimination of gas, air, or foam from the gastro-intestinal tract prior to radiography, and for the relief of abdominal distension and dyspepsia. It is often used in conjunction with antacids such as aluminium hydroxide.

Pharmacokinetic properties:

The insoluble aluminium compounds that constitute aluminium hydroxide are slowly but perhaps incompletely solubilised in the stomach to form aluminium chloride. Some absorption of aluminium from the soluble salt occurs in the gastro-intestinal tract with some excretion in the urine. Some unabsorbed aluminium hydroxide combines with phosphates present in the gut to form insoluble aluminium phosphates and some forms other insoluble salts such as carbonates and salts of fatty acids; all these salts are excreted in the faeces.

Approximately one third of magnesium is absorbed from the small intestine following oral absorbed from the small intestine following oral administration and even soluble magnesium salts are generally very slowly absorbed. Absorption of magnesium is enhanced by the presence of the vitamin D compound calcitriol. Magnesium salts are excreted mainly in the urine with very limited faecal loss; small amounts are excreted in breast milk and saliva. Magnesium also crosses the placenta.

Activated dimethicone and alginic acid have nil /negligible systemic absorption and act topically.

Indication:

It is indicated in the relief of symptoms of gastro-oesophageal reflux such as acid regurgitation, heartburn and indigestion, for example following meals or during pregnancy, and for symptoms of excess stomach acid (hyperacidity).

Posology and Route of Administration:

For Tablet:

For oral administration, after being thoroughly chewed.

Adults and children 12 years and over: Two to four tablets after meals and at bedtime, up to four times per day.

Children under 12 years: Should be given only on medical advice.

Elderly: No dose modifications necessary for this age group.

Route of administration:

Oral

Contra indications:

Hypersensitivity to any of the ingredients.

Side effects and Adverse Reactions:

Magnesium salts are relatively poorly absorbed following oral administration, but in patients with impaired renal function there may be sufficient accumulation to produce toxic effects. Excessive administration of magnesium leads to the development of hypermagnesaemia. Symptoms of hypermagnesaemia may include nausea, vomiting, flushing of the skin, thirst, hypotension due to peripheral vasodilation, drowsiness, confusion, loss of tendon reflexes due to neuromuscular blockade, muscle weakness, respiratory depression, cardiac arrhythmias, coma and cardiac arrest. Acute ingestion of magnesium-containing compounds may also cause gastro-intestinal irritation and watery diarrhoea. Aluminium hydroxide is an astringent and may cause nausea, vomiting, and constipation; large doses can cause intestinal obstruction. Excessive doses, or even normal doses in patients with low-phosphate diets, may lead to phosphate depletion accompanied by increased resorption and urinary excretion of calcium with the risk of osteomalacia. Osteomalacia, and also encephalopathy and dementia, have occurred in patients with chronic renal failure who received relatively high doses of aluminium hydroxide as a phosphate-binding agent.

Interaction with other medicaments:

Magnesium salts and aluminium hydroxide may interfere with the absorption of other drugs when these are taken concomitantly.

Warning and Precautions:

Due to its aspartame content this product should not be given to patients with phenylketonuria.

If symptoms do not improve after seven days, the clinical situation should be reviewed.

Treatment of children younger than 12 years of age is not generally recommended, except on medical advice.

Pregnancy and lactation:

No data available on pregnant and nursing mother.

Over dosage:

In the event of over dosage, symptomatic treatment should be given. The patient may notice abdominal distension.

Shelf life:

30 months from the date of manufacture.

Storage condition:

Megacid Tablet : Store below 30°C,
keep away from the sunlight.

Packing and Pack sizes:

Megacid Tablet - 10 tablets in a blister and 3 such blisters in a carton

Manufactured by:

Sai Mirra Innopharm Pvt. Ltd

No. 288 & 299, SIDCO Estate,
Ambattur, 600 098 Chennai, India.

Product Registration Holder

and Imported by:

Unimed Sdn Bhd,

No.53, Jalan Tembaga SD 5/2B, Bandar Sri Damansara,
52200 Kuala Lumpur, Malaysia.

Revision date : 21 January 2025.

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